



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رابعياً : شوفيه محمد السيد علي

العنوان : 0

المحافظة : اسوان

الحى / المدينة : ادفو

رقم البطاقة : 26712112100707

رقم التليفون : 01153952395

العمر (تاريخ الميلاد) : 11-12-1967 المهنة : بدون عمل

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

خطاب إداري

هذا الرقم القومي يتبع منظومة التأمين الصحي الشامل جهة الاختصاص



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Name: شوفيه محمد السيد علي National ID: 26712112100707	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

شوقيه

محمد سيد على

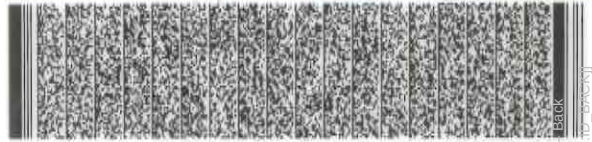
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التبين - القاهرة



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National ID - Front
[[DOC]] RE-ID_FRONT



Name : Shawqiya Mohammed Said Ali

Date : 15-08-2026 01:46

Branch : Maadi

Medical No. : 439901

Ref.Physician Prof : Islam Hussain

Ins.Company : شركة مياه الشرب للقاهرة الكبرى

DIGITALIZED LOW DOSE SOFT TISSUE MAMMOGRAPHY WITH COMPLEMENTARY BREAST US EXAMINATION OF BOTH BREASTS REVEALED:

- A well defined speculated hyperdense lesion is seen at the LIQ of the left breast corresponding by US to well defined globular shaped heterogeneous hypoechoic lesion at 8 o'clock position , surrounded by echogenic halo and causing posterior shadowing , subcutaneous infiltration. It measures about 22x15 mm in its max dimensions with about 4.5 cm distance from adjacent nipple .
- left axillary tail enlarged lymph node, with cortical thickening and lost fatty hila , measures 19x16 mm .
- ACR B: Mixed fibro-fatty-glandular breast parenchyma.
- No pathological microcalcification clusters.
- No right breast solid or cystic lesions. with Clear lumpectomy bed
- Normal central ducts system.
- Normal skin thickness and contour as well as right nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- ✓ Left breast LIQ malignant looking lesion (BIRADS V) associated with ipsilateral axillary lymphadenopathy .
- ✓ Clear Right breast lumpectomy bed

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0 : Needs further investigations.

BIRADS 1: Normal findings.

BIRADS 2: Benign.

BIRADS 3: Probably benign.

BIRADS 4: Probably malignant.

BIRADS 5: Malignant.

BIRADS 6: Proven pathologically malignant

Best regards

Dr Mahmoud Abdelmohsen.FEBR.FABR

Patient Name	شوقيه محمد سيد على	Case No.	359981
Sex/Age	Female 58 Y	Patient No.	26142
Registered Date	23/8/2026 02:28:02 PM	Referred By Dr.	Interventional Radiology
Validation Date	07/9/2026 01:06:37 PM	Branch	Gustave Roussy
Reporting Date	19/9/2026 10:41:09 AM	Pathology No.	2673/26

Pathology Report

Clinical Data: Left LIQ breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 20/08/2026, at 03:10 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Four grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma with micropapillary features.
 - **Histologic grade:** 2 (differentiation 2, pleomorphism 2, mitosis 2).
 - **DCIS:** Absent.
 - **Microcalcification:** Absent.
 - **Angioinvasion:** Suspicious.
 - **Perineural invasion:** Absent.

Results:

* IPT (All controls showed appropriate reactivity):
 - ER: Moderate nuclear reaction in 10% of tumor cells (score 4/8).
 - PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).
 - Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
 - Ki67%: 45%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2 with micropapillary features.

IPT:

- ER: Positive (score 4/8).
 - PR: Negative (score 2/8).

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- Her2: Positive (score 3+).
- Ki67%: 45%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei, MD, PhD.
Dr. Walaa Ibrahim Ghoneim, MD.